Whole back pain assessment

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This tool supports decisions about how soon to seek care and which specialist to see. It does not diagnose or treat, and it does not replace an examination by a doctor. If you feel very unwell or are worried, go to a hospital.

See a spine specialist within a week

Not an emergency, but your symptoms need a proper examination. Go sooner if the pain worsens, numbness spreads or a leg feels weak.

- Pain in more than one part of the spine
- This bout has lasted more than 6 weeks
- Inflammatory pattern: long morning stiffness, better with exercise
- Rounded back, curve or height loss
- Pain disturbing sleep

CLINICAL IMPRESSION

Most likely: Inflammatory spine disease affecting the whole back (ankylosing spondylitis) [Strong match]

Axial spondyloarthritis

Supports: Pain in more than one spinal region · Stiff for more than 30 minutes after waking · Exercise and movement · Psoriasis, inflammatory bowel disease or recurring red, painful eye · Wall test (posture): positive · Resting (better once you move) · Wakes you late at night and eases after getting up

Inflammation starts in the sacroiliac joints and lower back and can climb the whole spine to the neck, causing morning stiffness that eases with exercise and, over years, a stooped posture.

Why your answers point here

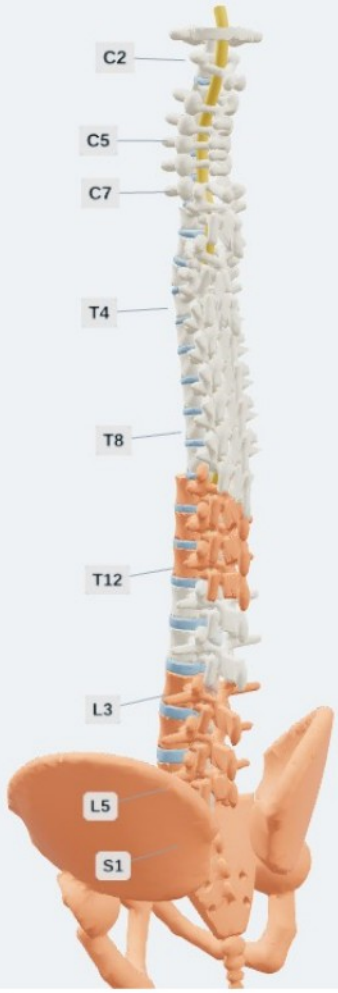
- Pain in more than one spinal region — Pain in two or more parts of the spine suggests a whole-spine process such as wear at several levels or inflammation.
- Stiff for more than 30 minutes after waking — Morning stiffness over 30 minutes is a feature of inflammatory back pain.
- Exercise and movement — Pain that eases with exercise suggests inflammation rather than injury.
- Wall test (posture): positive — Not touching the wall with the head shows a rounded upper back.
- Resting (better once you move) — Pain worse at rest suggests inflammation.
- Wakes you late at night and eases after getting up — Waking late at night and easing on getting up is typical of inflammatory back pain.
- This bout has lasted more than 3 months without going away — Pain beyond 3 months is chronic and needs a structured plan.

Why it happens: An immune condition linked to HLA-B27, usually starting before 40; psoriasis, bowel inflammation and eye inflammation are related.
How doctors confirm it: Blood tests (CRP, ESR, HLA-B27) and MRI of the sacroiliac joints and spine.
How it is treated: Daily exercise, anti-inflammatory medicines and biologics under a rheumatologist.
Outlook: Early treatment preserves movement and posture.

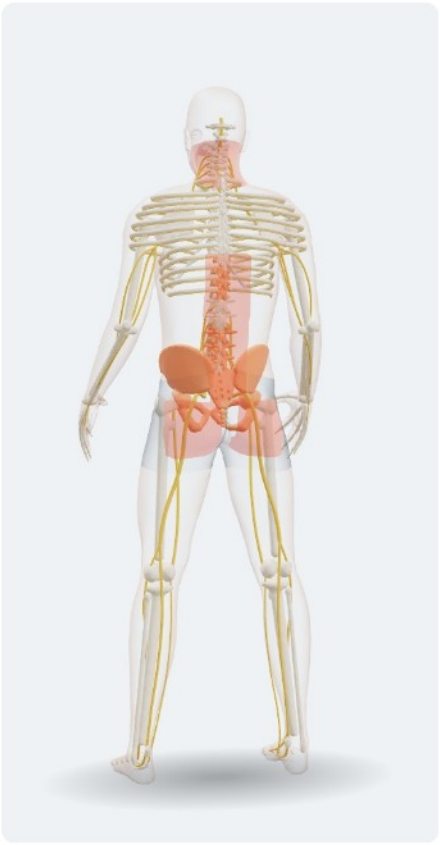
A DOCTOR'S EXAMINATION IS ESSENTIAL

- This assessment cannot test your reflexes, muscle strength or skin sensation, which show which nerve is involved and how badly.
- Only a doctor can decide whether you need an MRI, X-ray or blood tests, and interpret them alongside your examination.
- Similar pain can come from the hip, kidney, pelvis or abdomen; a doctor examines these too.
- Medicines, injections and exercise plans must be chosen for you personally.

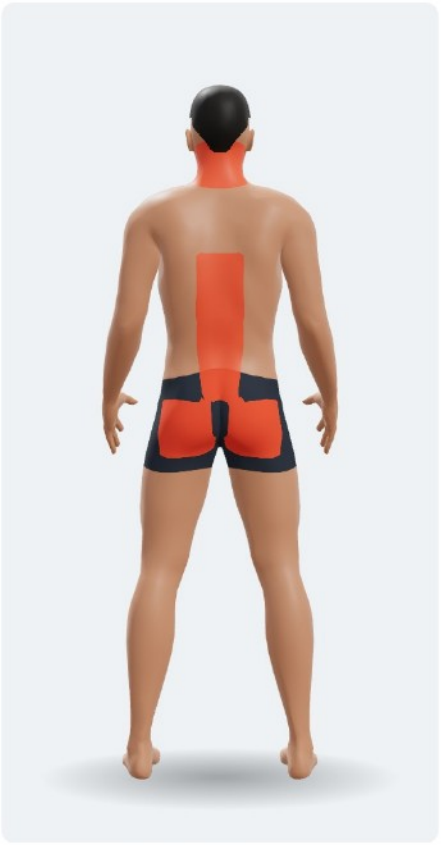
3D ANATOMY OF YOUR PROBLEM



The whole spine from the neck to the tailbone, for reference.



Full skeleton and nerves with the areas you marked.



Areas you marked, back view



Areas you marked, front view

OTHER POSSIBILITIES

• Wear in several parts of the spine (multilevel spondylosis) [Possible] — Pain in more than one spinal region, This bout has lasted more than 3 months without going away
• Spinal curve (scoliosis or kyphosis) [Possible] — Wall test (posture): positive, This bout has lasted more than 3 months without going away, Pain in more than one spinal region
• Osteoporosis with spinal fractures [Possible] — Wall test (posture): positive, Middle back, Pain in more than one spinal region
• Postural and muscle pain in several areas [Possible] — Pain in more than one spinal region, Exercise and movement, 18–29

WHICH SPECIALIST TO SEE

1. Rheumatology (Start here)
Inflammatory arthritis of the spine; blood tests and SI-joint MRI. For: Inflammatory spine disease affecting the whole back (ankylosing spondylitis).

2. Physiotherapy / PM&R
Structured exercise and rehabilitation; the core of treatment for most back pain. For: Inflammatory spine disease affecting the whole back (ankylosing spondylitis).

YOUR ANSWERS

Age	18–29 · Male
Medical history (tick any that apply)	Psoriasis, inflammatory bowel disease or recurring red, painful eye
Areas marked	Neck; Middle back; Lower back; Hip and buttock (both sides)
How did it start?	Slowly, with no clear reason

When did it start?	This bout has lasted more than 3 months without going away
How does the pain behave?	Stiff for more than 30 minutes after waking; Exercise and movement; Resting (better once you move); Wakes you late at night and eases after getting up
Impact on daily life	Pain right now 5/10 · Walking: Normally · Sleep: Pain wakes me up
Wall test (posture)	Positive
Chair rise test	Not answered
Red-flag screen	No to all warning signs

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